

§ 170.315(a)(3)

Computerized provider order entry (CPOE) – diagnostic imaging

Certification Companion Guide v1.0

Issued Date: 03-11-2024

This Certification Companion Guide (CCG) is an informative document designed to assist with health IT product certification. The CCG is not a substitute for the requirements outlined in regulation and related ONC final rules. It extracts key portions of ONC final rules' preambles and includes subsequent clarifying interpretations. To access the full context of regulatory intent please consult the [Certification Regulations](#) page for links to all ONC final rules or consult other regulatory references as noted. The CCG is for public use and should not be sold or redistributed.

Certification Clarifications

Technical Explanations and Clarifications

- There is no standard required for this certification criterion.
- To meet the Base electronic health record (EHR) definition, providers must possess technology that has been certified to at least one of the following: § 170.315(a)(1) Computerized provider order entry (CPOE) – medications, § 170.315(a)(2) Computerized provider order entry (CPOE) – laboratory, or § 170.315(a)(3) Computerized provider order entry (CPOE) – diagnostic imaging.

Technical outcome – The health IT permits a user to record, change, and access diagnostic imaging orders.

- No standard is required for demonstrating the ability to allow a user to record, change, and access diagnostic imaging orders.
- This provision does not focus on the transmission of diagnostic imaging orders, only on the ability of a user to record, change, and access the diagnostic imaging order. [see also [77 FR 54248](#)]

Technical outcome – The health IT allows for the user to include a “reason for order.”

- It is not mandatory that the health IT allow a user to include a “reason for order” field. However, it is optional. The developer has the discretion to determine how to implement this optional provision (e.g., free text field or drop-down menu of pre-determined entries).

Regulation Text

§ 170.315 (a)(3) Computerized provider order entry—diagnostic imaging—

- (i) Enable a user to record, change, and access diagnostic imaging orders.
- (ii) Optional. Include a "reason for order" field.

Standards & References

The following standards are referenced within the certification criteria for:

§ 170.315(a)(3) Computerized provider order entry (CPOE) – diagnostic imaging

Certification Dependencies

Privacy and Security

This certification criterion was adopted at § 170.315(a)(3). As a result, an ONC Authorized Certification Body (ONC-ACB) must ensure that a product presented for certification to a § 170.315(a) criterion includes the privacy and security criteria (adopted in § 170.315(d)) within the overall scope of the certificate issued to the product.

- The privacy and security criteria (adopted in § 170.315(d)) do not need to be explicitly tested with this specific paragraph (a) criterion unless it is the only criterion for which certification is requested.
- As a general rule, a product presented for certification only needs to be presented once to each applicable privacy and security criterion (adopted in § 170.315(d)) so long as the health IT developer attests that such privacy and security capabilities apply to the full scope of capabilities included in the requested certification. However, exceptions exist for § 170.315(e)(1) “View, download, and transmit to 3rd party (VDT)” and (e)(2) “Secure messaging,” which are explicitly stated.
- § 170.315(d)(2)(i)(C) is not required if the scope of the Health IT Module does not have end-user device encryption features.

For more information on the approaches to meet these Privacy and Security requirements, please download the Privacy and Security CCG below:

Approaches:

If choosing

Approach 1

- [Authentication, access control, and authorization \(§ 170.315\(d\)\(1\)\)](#)
- [Auditable events and tamper-resistance \(§ 170.315\(d\)\(2\)\)](#)
- [Audit reports \(§ 170.315\(d\)\(3\)\)](#)

If choosing

Approach 2

For each applicable privacy and security certification criterion not certified for Approach 1, the health IT developer may certify using system documentation that is sufficiently detailed to enable integration such that the Health IT Module has

- [Amendments \(§ 170.315\(d\)\(4\)\)](#).
- [Automatic access time-out \(§ 170.315\(d\)\(5\)\)](#).
- [Emergency access \(§ 170.315\(d\)\(6\)\)](#).
- [End-user device encryption \(§ 170.315\(d\)\(7\)\)](#).
- [Encrypt authentication credentials \(§ 170.315\(d\)\(12\)\)](#).
- [Multi-factor authentication \(MFA\) \(§ 170.315\(d\)\(13\)\)](#).

implemented service interfaces for each applicable privacy and security certification criterion that enable the Health IT Module to access external services necessary to meet the requirements of the privacy and security certification criterion. Please see the *21st Century Cures Act: Interoperability, Information Blocking, and the ONC Health IT Certification Program Final Rule* at [85 FR 25710](#) for additional clarification.

Design and Performance

The following design and performance certification criteria (adopted in § 170.315(g)) must also be certified in order for the product to be certified.

- [Safety-enhanced design \(§ 170.315\(g\)\(3\)\)](#) must be explicitly demonstrated for this criterion.
- [Quality management system \(§ 170.315\(g\)\(4\)\)](#): When a single quality management system (QMS) is used, the QMS only needs to be identified once. Otherwise, the QMS' need to be identified for every capability to which it was applied.
- [Accessibility-centered design \(§ 170.315\(g\)\(5\)\)](#): When a single accessibility-centered design standard is used, the standard only needs to be identified once. Otherwise, the accessibility-centered design standards need to be identified for every capability to which they were applied; or, alternatively, the developer must state that no accessibility-centered design was used.

Certification Companion Guide Changelog

The following changelog applies to:

§ 170.315(a)(3)Computerized provider order entry (CPOE) – diagnostic imaging

Revision History

Version

1.0

Description of Change

Initial Publication

Version Date

03-11-2024

